Who Pays After Medicaid Expansion?

Coverage, spending and utilisation across payers in US states, 2010–2018

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The question

Medicaid expansion raised Medicaid enrollment and Medicaid spending. What it did to *everyone else* is less settled — whether the new coverage displaced private insurance, and whether the new spending displaced anyone else's.

We follow coverage, spending and utilisation for every major payer through the same design, so the answers have to add up.

Data and design

IHME Disease Expenditure Project (DEX 2.0): state-level spending and encounters by payer and setting, 2010–2018. Uninsurance from CPS.

Difference-in-differences comparing the **25 states that expanded on 1 January 2014** with the **19 that never expanded**. Callaway and Sant'Anna (2021) group-time estimator, never-treated controls, adjusting for poverty, median income, pre-expansion uninsurance and age structure. Standard errors clustered by state, multiplier bootstrap.

Enrollee counts and population are recovered from the reported ratios, so coverage is measured inside the same source as spending. On the all-ages basis all four payers return the same population to within **0.005%**.

Later cohorts are excluded: they contain 3, 3 and 1 states, and at that size a placebo test rejects for a one-state cohort 88% of the time.

Spending has three parts

$$\frac{\text{spend}}{\text{resident}} = \frac{\text{spend}}{\text{encounter}} \times \frac{\text{encounters}}{\text{enrollee}} \times \frac{\text{enrollees}}{\text{resident}}$$

Price, use, and coverage. An identity, so each margin is estimated separately and read against the others.

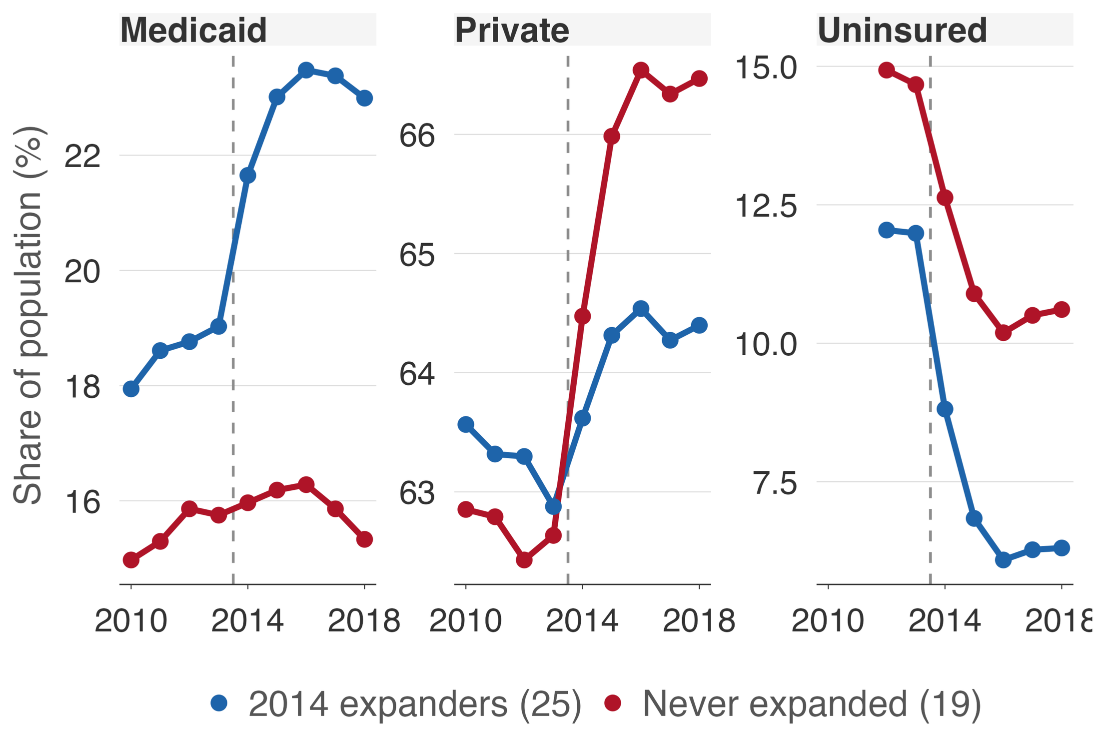
Limitations

Estimates describe 2014 expanders only.

Out-of-pocket spending is not analysed. DEX defines it as insured cost-sharing plus all spending by the uninsured, and expansion changes that mix directly. Its pre-trend test fails in every setting.

Private coverage changes cannot be attributed to individual switching: this design identifies group differences, not flows.

1. Expansion covered the uninsured



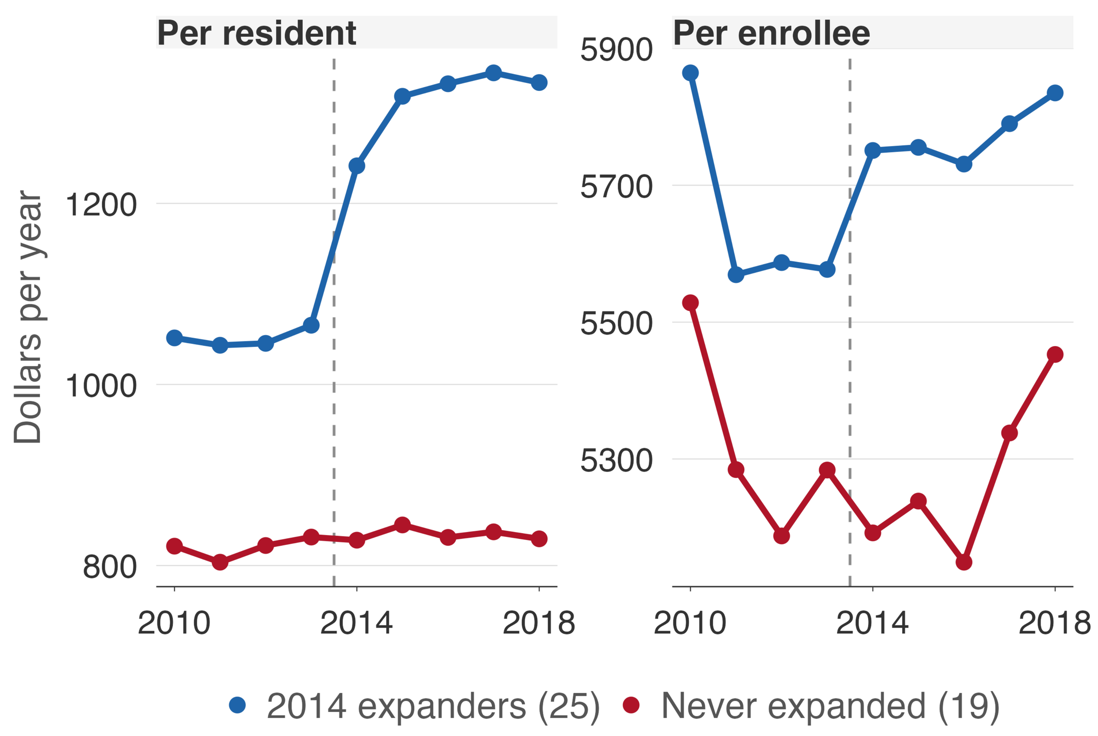
Blue: the 25 states that expanded in 2014. Red: the 19 that never expanded.

Coverage	Effect	95% CI	Pre-test
Medicaid	+3.69	[+2.16, +5.23]	0.84
Medicare	−0.20	[−0.54, +0.14]	0.72
Uninsured	−1.38	[−2.47, −0.30]	0.30
Private	−2.16	[−3.04, −1.27]	0.96

Medicaid rose **3.7 points**. The uninsured rate fell **1.4 points** further than in control states.

Private coverage rose in both groups — 62.9 to 64.4% in expanders, 62.6 to 66.5% in controls. The negative estimate is control states gaining faster, not expansion states losing. Marketplace subsidies cover 100–138% FPL where Medicaid does not.

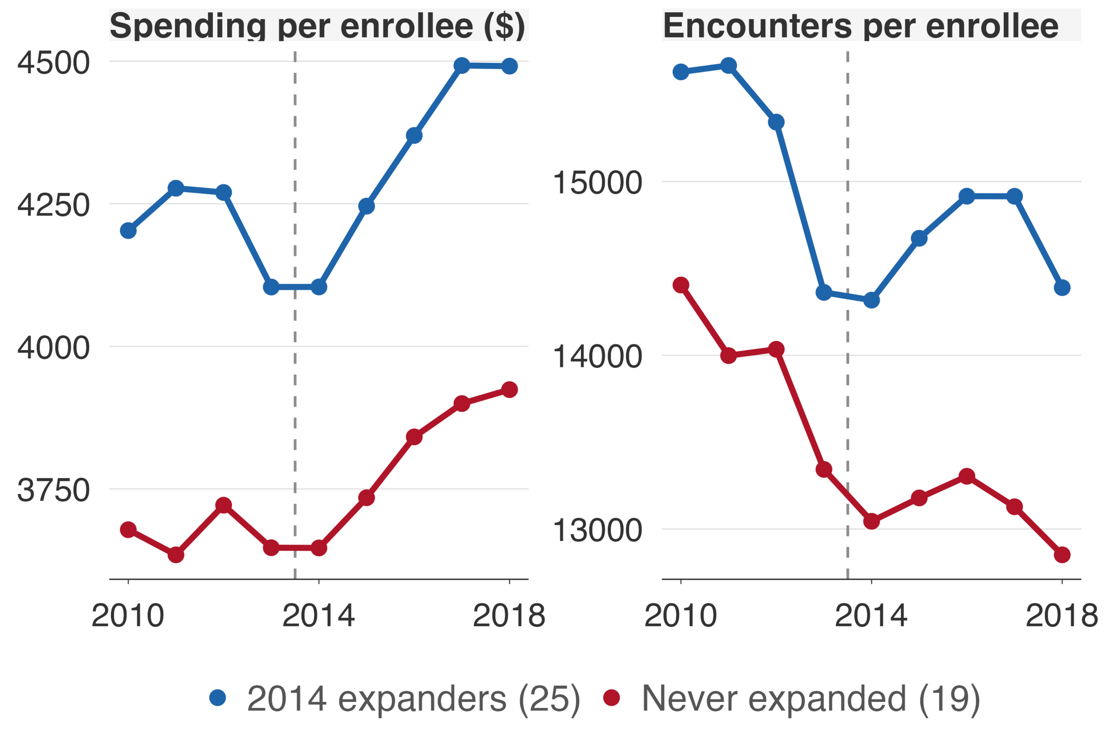
2. Spending followed enrollment, not intensity



Blue: expansion states' actual path. Grey dashed with shading: the estimated counterfactual with a 95% band. Before 2014 the band is the parallel-trends test. (Delete this note when using the `lines` style — the legend covers it.) Medicaid spending per resident rose **18.0%** (SE 4.0). Per enrollee it did not move: **−3.3%** (SE 4.2).

Solving the identity, a new enrollee costs about **80%** of an incumbent (95% CI 0.29 to 1.31). The expansion population was slightly cheaper than the existing caseload, not dramatically so.

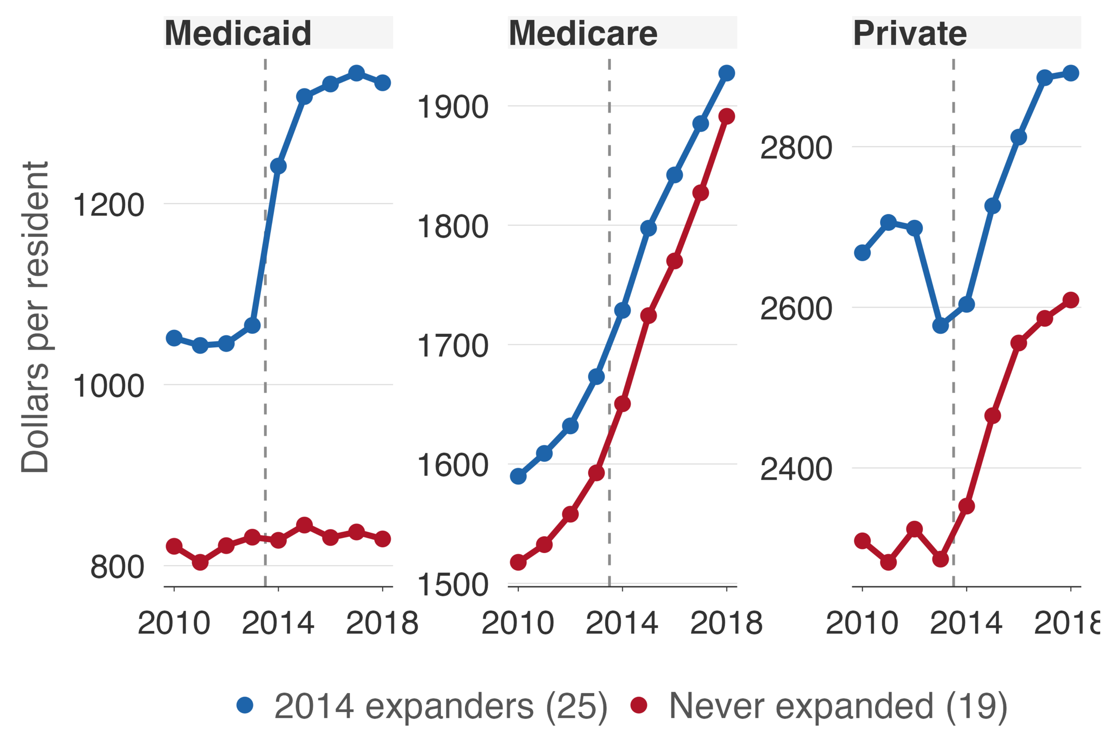
3. Weak signs of a changed private pool



Private spending per enrollee **+4.8%** (SE 2.8) and encounters per enrollee **+3.3%** (SE 2.4). Neither reaches significance.

Because both margins move together, this is **use, not price**. The most likely reading is dilution of the *control* group's risk pool as lower-utilising marketplace enrollees joined it.

4. No spillover to other payers



Medicare spending per resident **+0.4%** (SE 1.3) — nothing, on any margin. Private spending per resident **+1.7%** (SE 2.5).

Across **84 estimates** covering four payers, five settings and four margins, every result significant at 5% is a Medicaid result. Among the 18 non-Medicaid spending models, **none** is significant — chance alone would give about one.

Every spending estimate

Spending per resident

Setting	Medicaid	Medicare	Private
All care	+18.0 (4.0) *	+0.4 (1.3)	+1.7 (2.5)
Ambulatory	+23.6 (6.4) *	−0.9 (1.1)	+2.1 (2.5)
Emergency	+21.7 (8.3) *	+2.9 (4.8)	−0.2 (1.8)
Inpatient	+22.5 (4.7) *	+0.5 (1.3)	+1.2 (3.3)
Nursing	+2.0 (6.7)	+3.0 (5.6)	+4.6 (6.8)
Pharmacy	+11.1 (12.4)	+0.8 (3.1)	+0.8 (5.0)

Spending per enrollee

Setting	Medicaid	Medicare	Private
All care	−3.3 (4.2)	+1.5 (0.9)	+4.8 (3.0)
Ambulatory	+0.0 (6.1)	+0.9 (0.9)	+5.3 (3.1)
Emergency	−2.0 (7.6)	+3.8 (3.0)	+3.2 (2.2)
Inpatient	+4.4 (5.1)	+0.8 (1.0)	+4.0 (3.7)
Nursing	−14.4 (9.4)	+4.1 (4.8)	+7.3 (7.2)
Pharmacy	−22.5 (17.7)	+3.2 (1.4) *	+4.2 (4.4)

Expansion moved people onto Medicaid and money followed them, one enrollee at a time.

Spending per enrollee did not move, Medicare did not move, and private insurance grew in expansion states too — just more slowly than where the marketplace did the covering.

Effect as % of the pre-2014 level (SE). * $p < 0.05$. Every Medicaid model passes the parallel-trends pre-test; private is borderline at $p = 0.07$.

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